THE MATERNITY METHOD

NCLEX CHEAT SHEET

Gestational Diabetes

Maternal & Newborn › Antepartum Complications

The Big Picture

Gestational diabetes (GDM) = high blood sugar that **first shows up in pregnancy**. Placental hormones make mom **resistant to insulin**, mostly in the **2nd–3rd trimester**. Sugar crosses the placenta, insulin doesn't, so **baby makes extra insulin**, grows **big**, and can **crash** into low blood sugar after birth.

KNOW THE NUMBERS

24–28 wks	< 95	< 140 / < 120	> 4,000 g	4–12 wks
Screen everyone	**Fasting** glucose goal (mg/dL)	**1-hr / 2-hr** after-meal goal	**Macrosomia** (about 9 lb)	Postpartum **glucose test**

Who's at Risk

- **Overweight** or obesity
- **GDM** in a past pregnancy
- Past baby **> 4,000 g**
- **Family history** of type 2 diabetes
- **PCOS**, older age, high blood pressure

Screening

- **2-step:** 1-hr 50 g glucose challenge (no fasting) → if high, **3-hr 100 g OGTT** (fasting). **2+ high values = GDM**
- **1-step:** 2-hr 75 g OGTT (fasting); **1 high value = GDM**
- High-risk patients may be screened at the **first visit**

Why It Matters

Risks for Baby

- **Macrosomia** → **shoulder dystocia**, birth injury
- **Low blood sugar** after birth
- **Breathing trouble** (insulin slows surfactant)
- Jaundice, low calcium, stillbirth

Risks for Mom

- **Preeclampsia**
- **Cesarean** birth
- Extra amniotic fluid (polyhydramnios)
- **Type 2 diabetes** later in life

Nursing Actions

Goal: **keep sugars in range** + **safe baby**

STEP 1

Diet & Move

- **Meal plan first**: carbs spread over **3 meals + 2–3 snacks**, with a **bedtime snack**
- **Walk after meals**; about 30 min most days
- Check glucose **fasting + after meals** and log it

STEP 2
Medicine

- **Insulin is first choice**: it **doesn't cross** the placenta
- Metformin may be used (it does cross)
- Insulin needs **rise** through the 2nd–3rd trimester and **drop fast** once the **placenta is out**

STEP 3
Labor & After

- **Hourly glucose** in labor; IV insulin/dextrose to keep about 70–110
- Most stop insulin **after birth**; breastfeeding helps
- **75 g OGTT at 4–12 wks postpartum**, then screen every 1–3 years

MEMORY HACK · LOW SUGAR IN MOM

Shaky, sweaty, confused? Rule of 15

15 g	15 min	Repeat	Snack
fast carbs (4 oz juice, glucose tabs)	wait, then **recheck**	if still < 70	with protein once it's up

Baby of a Diabetic Mom

- **Check glucose early**, often around 30 min after the first feed and before feeds (per protocol)
- **Low sugar signs: jittery**, poor feeding, **low temp**, lethargy, high-pitched cry, apnea, seizures
- **Treat:** feed early and often (breast or formula), **dextrose gel**; IV dextrose if severe or not improving
- Check for **birth injury** after a big baby (clavicle, arm movement)

RED FLAGS · STOP AND CALL THE PROVIDER

Mom's glucose **< 70** with symptoms · Newborn **jitteriness, apnea, or seizure** · Newborn won't feed · Signs of **preeclampsia** (headache, vision changes, BP ≥ 140/90)

MUST-KNOW FOR THE NCLEX

- **Screen at 24–28 weeks.**
- **Diet and exercise first**; **insulin** is the preferred medication.
- Insulin needs **drop right after the placenta** delivers. Watch for maternal lows.
- Baby is at risk for **hypoglycemia**, **macrosomia/shoulder dystocia**, and **breathing trouble**.
- Mom needs a **glucose test 4–12 weeks postpartum**.

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QUICK CHECK · TEST YOURSELF

1. Routine GDM screening happens at ____–____ weeks.
2. The preferred medication for GDM is _____ because it doesn't cross the placenta.
3. A jittery newborn of a diabetic mother needs a _____ check first.
4. Insulin needs drop suddenly after the _____ is delivered.
5. A big baby (> 4,000 g) raises the risk of _____ _____ during birth.

Answers: 1. 24–28 · 2. insulin · 3. blood glucose · 4. placenta · 5. shoulder dystocia

For study use only. Always follow facility protocols, current guidelines, and provider orders.

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